Narrated Accessibility Walk

Massachusetts General Hospital area, Boston, MA: Charles/MGH station, Cambridge St, 50 Staniford St, 101 Merrimac St and the Main Entrance (55 Fruit St)

Pilot report 2 | Recorded October 8, 2026 | Revised October 25, 2026
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Summary

Two narrated smartphone videos and one narrated audio recording documented the journey a patient or research participant might take from the Charles/MGH transit station to Massachusetts General Hospital buildings: an outpatient clinic building, a nearby medical office building, and the hospital's Main Entrance, the building that houses its clinical research center for study visits. Items are possible barriers and positive features observed along the way.

- **20 items recorded:** 5 rated high severity, plus several positive features.
- **Accessible options exist but are not always found or usable without help.** At the Main Entrance, an older adult with a walker struggled through a revolving door while a wide automatic door stood nearby. Inside an outpatient building, a wheelchair user could pass a narrow clinic door only by being pushed.
- **The route itself is the main burden:** broken bricks around utility lids, damaged crosswalks, a long and narrow construction walkway, and a badly damaged tactile warning pad at a major crossing.
- **Positives:** benches on the station platform and along Staniford St, public seating in a covered arcade, a ramp beside steps, handrails on a long ramp, push-button openers at entrances, and marked accessible parking.
- **Research recruitment:** a study recruitment sticker on a pedestrian signal pole at the Charles/MGH crossing offered only a QR code, with no phone number, web address or accommodation information.

Disclaimer. This report was produced with the help of AI tools (automatic speech transcription and AI-assisted image analysis; see "How this report was produced") and was reviewed by the author. Errors and omissions are possible. Items are possible barriers observed on one day, not legal compliance determinations or findings of violation. Distances and slopes are estimates unless stated otherwise.

People observed are described only in general terms. Observations reflect conditions on October 8, 2026 and may have changed since.

Walk details

Date	October 8, 2026, late morning to early afternoon
Recordings	Video A: Charles/MGH station to 50 Staniford St (11 min 30 s). Video B: 50 Staniford St to 101 Merrimac St (6 min 23 s). Audio C: approach to and observation at the MGH Main Entrance, 55 Fruit St (11 min 24 s).
Context	The walker traveled by subway to a scheduled outpatient appointment. After a minor skin procedure under local anesthetic, he felt somewhat dizzy and the numbness slightly affected his vision on the way back. This was a temporary post-procedure limitation; the walker does not have a disability. Many research visits also involve procedures, so participants may leave in a worse state than they arrived.
Narrator	Uri Kartoun, PhD (DBbun LLC); assessed the route from the point of view of wheelchair, walker and cane users, blind and low-vision people, and people who tire easily
Research relevance	Massachusetts General Hospital is among the busiest clinical research sites nearby: about 838 actively recruiting studies listed an MGH site on ClinicalTrials.gov (query of October 4, 2026).

Can people complete this journey?

From Charles/MGH station to the MGH Main Entrance (55 Fruit St, home of the Translational and Clinical Research Centers) and to an outpatient clinic building (50 Staniford St), by traveler group, based on the items below.

Traveler	Reach the buildings from the station?	Get inside to the visit?	Deciding barrier
Wheelchair (manual or power)	Yes, with difficulty: damaged crosswalks on Cambridge St; a long, narrow construction walkway (about 4 ft); the station elevator was not checked	Main Entrance: yes, through the wide automatic door, if it is noticed. Clinic at 50 Staniford St: only with help (narrow clinic door; the user had to be pushed through)	Narrow clinic door (item 10); construction walkway (item 5)
Walker or cane	Yes, with trip hazards: broken bricks around a utility lid, cracked crosswalks	With difficulty: an older adult with a walker struggled through a revolving door beside an unnoticed automatic door	Accessible door not obvious (item 17)
Blind or low vision	With risk: damaged tactile warning pad at a major crossing; garage driveways across the sidewalk; crowded corners	Yes, if guided; building numbers on Merrimac St not visible	Damaged tactile pad (item 13)
People who tire easily	Yes: benches on the station platform, on Staniford St and in the covered arcade; but no rest point along the construction walkway	Yes; drop-off congestion and slow transfers at the curb add waiting time	Construction walkway without rest (item 5)

Findings

Severity notes appear in the barrier column; “positive” marks features that worked well. Conf. (confidence): H = seen and recorded on video or photo; M = seen but not measured, or noted from audio only (items 17-20). Missing evidence for M items is typically a measurement (width, slope) or a video record.

A. Charles/MGH station to 50 Staniford St (video)

#	Where	Possible barrier or feature	Observation	Most affected	Conf.	Suggested fix
1	Charles/ MGH station	Stairs to street (elevator available)	Long stairs from the platform level to the street; an elevator exists but was not used or checked. Platform has benches and clear “Exit to hospitals” signs (positive).	Wheelchair, walker, energy-limited	M	Post elevator status at the platform and street; keep the elevator in service
2	Cambridge St crossings (incl. N. Grove St)	Damaged crosswalks; long signal waits	Cracked asphalt, potholes and utility covers inside crosswalks; worn paint; long waits, with many people crossing against the signal.	Wheelchair, walker, cane, low vision	H	Repave crosswalks; review signal timing
3	Cambridge St brick sidewalk	Broken bricks around a utility lid (High)	A large gap of missing and broken bricks around a utility box lid; heaved bricks elsewhere.	Wheelchair, walker, cane	H	Reset bricks and seal around the lid
4	Corner near new tower / constructio n	Street furniture narrowing the path	Mailbox, trash compactor, lamp post and granite blocks clustered at the corner.	Wheelchair, low vision	H	Consolidate or relocate furniture
5	Cambridge St constructio n zone	Long, narrow temporary walkway (High, temporary)	Covered scaffold walkway, then a walkway between concrete barriers, estimated at about 4 ft wide; uneven joints; no rest points along its length.	Wheelchair, scooter, walker, energy-limited	M	Maintain at least a 4-5 ft clear path; smooth joints; add a rest point
6	Blossom St crossing	Cracked, sloped crosswalk	Cracked crosswalk with a slope; flaggers directing traffic.	Wheelchair	M	Repave; keep a level crossing path
7	Charles River Plaza	Garage driveway across sidewalk	Vehicles cross the sidewalk at the garage entrance. Marked accessible parking spaces nearby (positive).	All, esp. low vision	H	Exit warning signal or mirror at the driveway
8	Charles River Plaza arcade (165 Cambridge St)	Steps with ramp; seating (positive)	Four steps with a ramp alternative; broken bricks at the ramp landing. The covered arcade offers public seating and a van/ambulance drop-off bay; seasonal displays and ladders narrow the arcade.	Wheelchair, low vision	H	Repair landing bricks; keep displays clear of the walking path
9	Ramp toward 50 Staniford St	Long ramp with handrails	Long ramp with handrails on both sides (positive); the walker perceived it as steep (not measured).	Manual wheelchair users, energy-limited	M	Measure slope; add an intermediate resting landing if needed

#	Where	Possible barrier or feature	Observation	Most affected	Conf.	Suggested fix
10	50 Staniford St entrance and clinic	Entrance with opener; narrow clinic door (High)	The building entrance appears to have a push-button opener (positive). Inside, the door into the clinic was narrow (estimated under 3 ft; not measured): a wheelchair user got through only by being pushed from behind, with very little clearance.	Wheelchair, scooter	M	Widen the door or add a powered door; usable without assistance

B. 50 Staniford St to 101 Merrimac St (video)

#	Where	Possible barrier or feature	Observation	Most affected	Conf.	Suggested fix
11	Sidewalk at 50 Staniford St	Service van on the sidewalk	A service van parked partly on the sidewalk/curb area narrowed the path. (A wheelchair user was observed moving easily along this sidewalk and slope.)	Wheelchair, low vision	H	Enforce no parking on sidewalks
12	Staniford St	Benches (positive); driveway crossing	Benches along a wide sidewalk. A pickup truck crossed the sidewalk leaving a garage.	Energy-limited (positive); all (driveway)	H	Driveway exit warning
13	Large intersection toward Merrimac St	Long crossing; damaged tactile pad (High)	Wide multi-lane crossing with bike lanes and faded paint. At the curb ramp, the tactile warning pad was badly damaged and peeling; a push button was present.	Blind and low vision; wheelchair (trip edge)	H	Replace the tactile pad; refresh crosswalk paint
14	Merrimac St	Hard to find the destination	Building numbers were not visible from the sidewalk; finding 101 Merrimac St took several minutes. A cracked concrete patch in the brick sidewalk near the entrance.	Cognitive, low vision, anyone fatigued	H	Large, visible street numbers; wayfinding sign; repair patch
15	101 Merrimac St entrance	Revolving door with automatic door beside it	Address plaque (positive). Main entry is a revolving door; an automatic swing door is beside it (“activate switch to operate” label). Switch location was not checked.	Wheelchair, walker, scooter	H	Confirm the switch is visible and within reach

C. Main Entrance, 55 Fruit St (audio observation)

#	Where	Possible barrier or feature	Observation	Most affected	Conf.	Suggested fix
16	Approach from Cambridge St	Visible signage; smooth sidewalk (positive)	Entrance signs visible from about 200 ft; smooth sidewalk; garage driveway and street crossing with cars stopping.	-	H	-

#	Where	Possible barrier or feature	Observation	Most affected	Conf.	Suggested fix
17	Main entrance, 55 Fruit St	Accessible door not obvious (High)	Four revolving doors plus a wide automatic door with a push button (estimated about 8 ft), set to the side. An older adult with a rolling walker struggled through a revolving door, apparently not noticing the automatic door.	Walker, wheelchair, low vision, cognitive	M	Clear signs pointing to the automatic door; place it in the main line of approach
18	Curbside drop-off	Congestion; slow transfers	Heavy car traffic at the curb. One transfer from a car into a wheelchair (fitting leg supports) took several minutes; a person of short stature needed the driver's help to climb into a high vehicle while its engine ran.	Wheelchair users and companions; people of short stature	M	Protected, sheltered transfer zone with time allowance
19	Near the entrance	Smoking near the entrance	Someone smoked about 30 ft from the entrance; strong smell.	Asthma, chemical sensitivity, MCAS	M	Enforce a smoke-free zone around entrances
20	Near the entrance	Abandoned walker	An abandoned walker stood near the entrance.	Low vision	M	Remove promptly

Research recruitment observed on the route

A recruitment sticker for a paid imaging study was posted on the pedestrian signal pole at the Cambridge St crossing next to Charles/MGH station, where people wait to cross toward the station. The only way to respond was a QR code. There was no phone number, web address, large-print or audio alternative, or statement about accommodations. People without smartphones, blind people and many older adults cannot respond, so the recruitment method itself screens them out.

Photo evidence

Frames were taken from the videos. Faces and other identifying details were blurred or are not identifiable.



Figure 1. Stairs from the Charles/MGH platform level toward the street (item 1).



Figure 2. Missing and broken bricks around a utility box lid on the Cambridge St sidewalk (item 3).



Figure 3. Covered scaffold walkway in the Cambridge St construction zone (item 5).

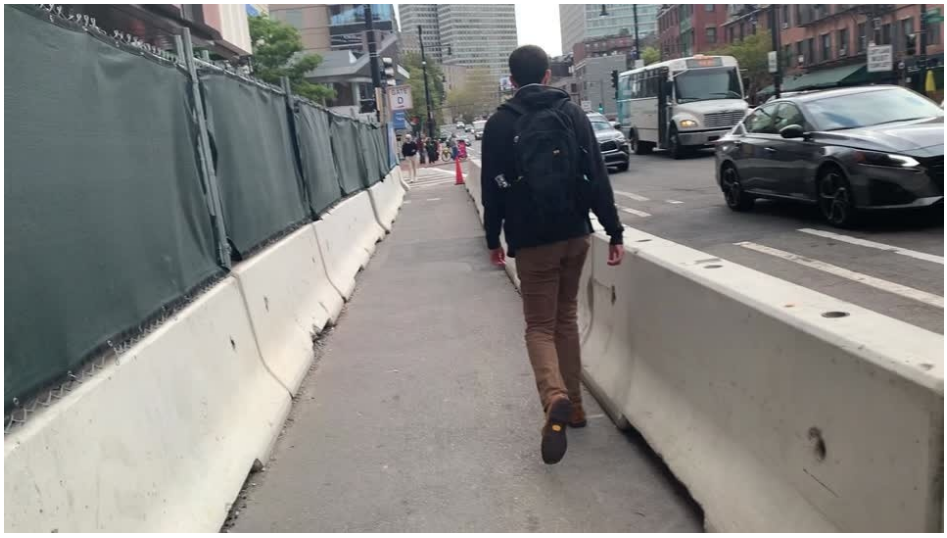


Figure 4. Temporary walkway between concrete barriers beside traffic (item 5).

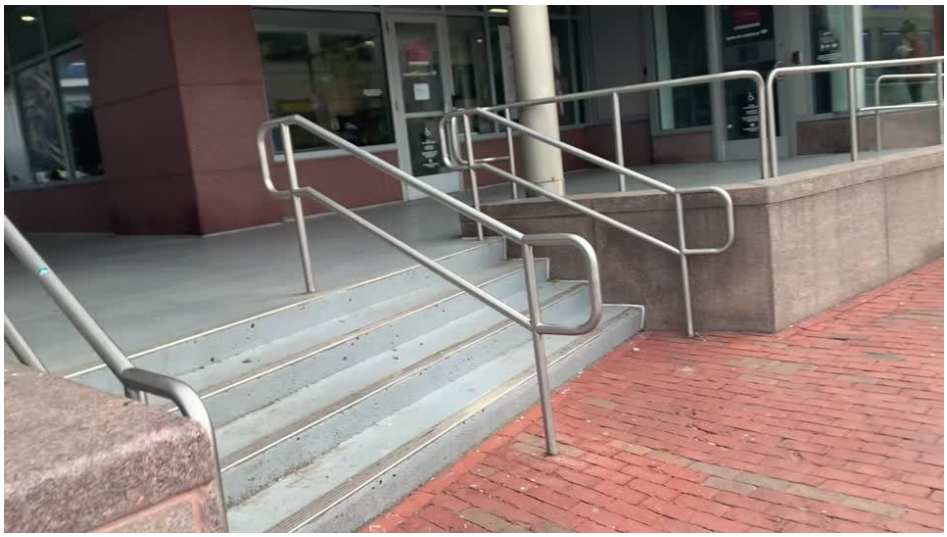


Figure 5. Steps with a ramp alternative at the Charles River Plaza arcade (item 8).



Figure 6. Public seating in the covered arcade, with a drop-off bay at left (item 8, positive).

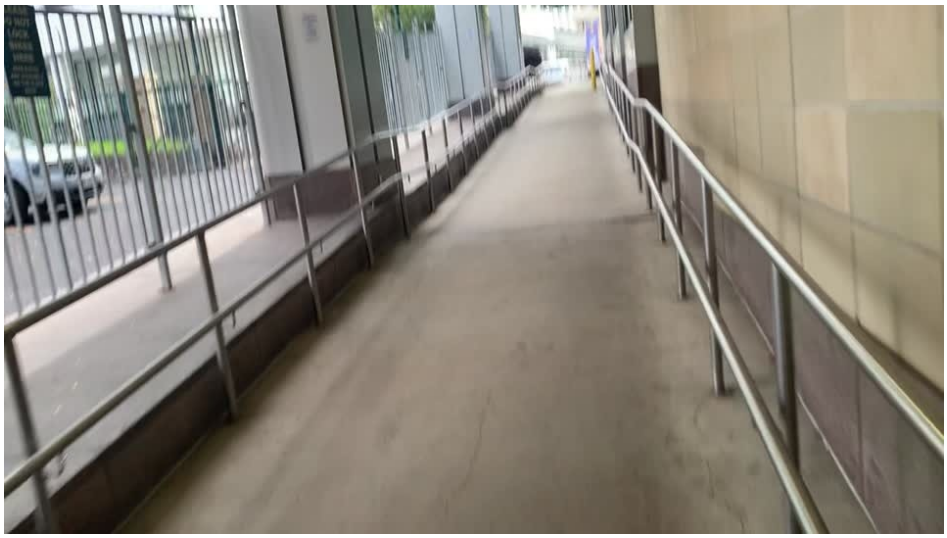


Figure 7. Long ramp with handrails on both sides toward 50 Staniford St (item 9).

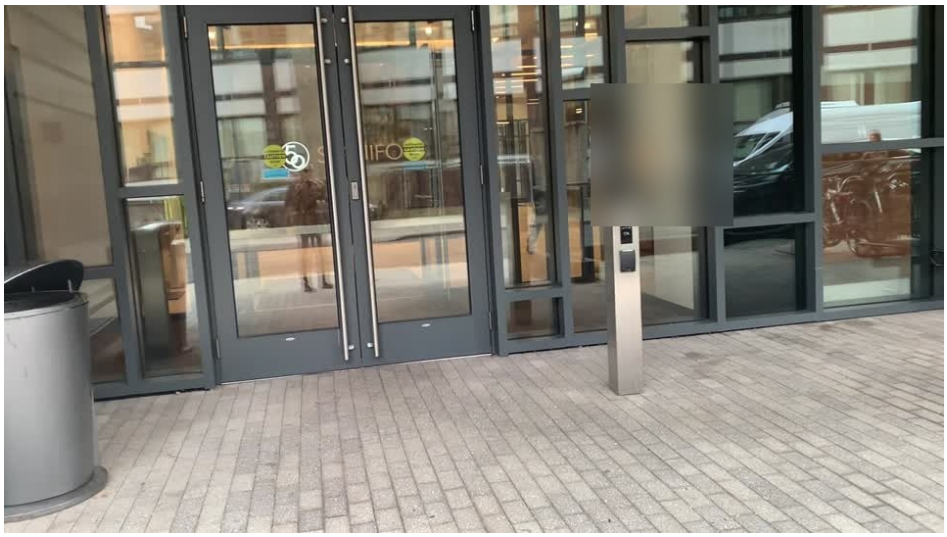


Figure 8. Entrance at 50 Staniford St with a push-button opener post (item 10).



Figure 9. Wide sidewalk with benches on Staniford St (item 12, positive).



Figure 10. Badly damaged, peeling tactile warning pad at a curb ramp (item 13).



Figure 11. Entrance at 101 Merrimac St with address plaque and revolving door (item 15).



Figure 12. Study recruitment sticker on a pedestrian signal pole near Charles/MGH station; the QR code and sponsor name are blurred in this report.

Checking the AI-assisted analysis

The item list was drafted with AI help from the transcribed narration and audio and from face-blurred video frames, then reviewed by the walker. The walker corrected one AI reading: the entrance at 101 Merrimac St was first drafted as having no alternative to the revolving door; a frame showing an automatic swing door with an “activate switch to operate” label changed it to a positive feature (item 15). The walker added one item observed indoors without recording (the narrow clinic door, item 10). Four items (17-20) rely on audio only and are rated M. The photos in this edition were re-extracted from the original videos on October 25, 2026, with faces blurred again.

An automated screen using only public data (City of Boston curb-ramp inventory, 311 requests, work zones, crash records and sidewalk inventory; OpenStreetMap; USGS elevation) was later run on the same routes. It partly indicated 8 of the 24 barriers counted in that comparison (33%) and missed 15, mainly day-of conditions such as the construction walkway, the unnoticed

automatic door and drop-off congestion. Public data helps plan a route; the walk finds what stops a participant on the day.

How this report was produced

- **Capture:** two narrated smartphone videos and one narrated audio recording, made by the author.
- **Local processing on the author's computer:** audio extracted with ffmpeg (imageio-ffmpeg 7.1); narration transcribed with faster-whisper, model medium.en, on a local GPU; one frame sampled every 3 seconds (358 frames); faces blurred with OpenCV 4.14 before any image left the computer; selected figures re-checked by eye.
- **AI review:** the blurred frames and the transcripts were reviewed by Claude (Anthropic), model claude-opus-5-5, which drafted the item list and this report. The author reviewed and corrected the result, including door features and locations, and added observations made inside the clinic building without recording.
- **Other data:** count of recruiting studies from the public ClinicalTrials.gov API (October 4, 2026).

Limitations

- One walker on one day; no measurements of door widths, slopes or crossing times.
- People were observed briefly in public spaces; their circumstances and needs are not known.
- Indoor observations were noted without recording and are approximate.
- Items are possible barriers, not legal compliance determinations.